Black Hills Orthopedic & Spine Center
7220 Mt. Rushmore Rd.
PO Box 6850 Rapid City, SD 57709
605-341-1414 Phone 605-341-7062 Fax

Patient:	**Viktoriya Pulliam**
Date of Birth:	**12/06/1989, Age 35**
Patient #:	**331748**
Visit Date:	**08/20/2025**
Attending Provider:	**Robert J. Woodruff MD**
Appt. Location:	**Black Hills Orthopedic & Spine of Wyoming**
Reason:	**New Spine Pain**

Patient Chart Note

Chief Complaint

The Chief Complaint is: Neck Pain.

History of Present Illness

Viktoriya Pulliam is a 35 year old female.

- Allergy list reviewed • Medication list reviewed

She is here for her initial evaluation with me and is referred by Dr. Ptacek. He has discussed placing a pain pump and wanted her evaluated to make sure that there is nothing else to be done prior to that. Her symptoms started in November 2019 when she was in a car accident. She was then in another auto accident a few months later. She ended up undergoing a C5-6 and C6-7 ACDF and this did help her symptoms although things have gradually been worsening over time. She gets symptoms in her neck and shoulders and diffusely down both arms although this does follow somewhat of a C7 distribution on the left. Her current symptoms are 4 out of 10 but can be 9 out of 10. She says that she has been worked up extensively including a neurologic workup for MS. This has been ruled out. She has been to the chiropractor and tried dry needling, Tylenol, ibuprofen, hydrocodone, muscle relaxants, Lyrica, and oral steroids. She says that the Lyrica makes her quite congested although it does help her symptoms. The baclofen also helps her symptoms as well but can make her somewhat forgetful. She is overall healthy with no chronic medical problems. She does not work. She is a stay-at-home mother with a toddler. She does not smoke. She is a social drinker. She does Pilates at home for exercise. Her current symptoms get in the way of activities of daily living such as bathing and showering and sleeping through the night. She has been told that she has OPLL and is concerned about that as well. She has had multiple opinions on her neck ranging from having her continue the medication to getting the pain pump. She has not had any epidural injections or other workup at this point.

Physical Findings

- Vitals taken 08/20/2025 11:39 am

BMI Education Provided	
Height	65.5 in
Weight	157 lbs 3.2 oz
Body Mass Index	25.8 kg/m2
Body Surface Area	1.8 m2

The patient is alert and oriented and answers questions appropriately about their history and is cooperative with examination. They appears to be their stated age. They do not appear to be uncomfortable. Cervical range of motion is full in all planes. There is no bruising, scarring, or swelling over the cervical spine. No shoulder impingement is noted on either side. There is 5 out of 5 deltoid, biceps, triceps, wrist flexion, wrist extension, grip, and intrinsic strength bilaterally. Sensation is intact in the C5-T1 distribution bilaterally. Tinel's is negative at the wrist and elbows bilaterally. Hoffmann's is negative bilaterally. Reflexes are symmetrical in the biceps, triceps, and brachioradialis. Distal pulses are palpable bilaterally.

Exam is as above except what is otherwise noted below:
Cervical range of motion is limited by 30% in extension and 20% in flexion and bilateral rotation, she has a right sided vertical cervical scar anteriorly

Tests
Educational Testing:

Questionnaires PROMIS:	Value
PROMIS Physical Function Short Form	21

I ordered and reviewed standing AP, lateral, flexion, and extension x-rays of the cervical spine from August 20, 2025. This shows anterior cervical instrumentation from C5-C7. There looks to be a solid arthrodesis at both those levels. There is moderate degenerative change at the C4-5 level. No instability is seen on flexion and extension views. No fractures are identified. Bone mineralization is unremarkable. When compared to x-rays from April 5, 2022, there has been progression in the degeneration predominantly at the C4-5 level but also mildly at C3-4.

I reviewed an MRI of the cervical spine from June 17, 2025. This was ordered by an outside provider and performed at an outside facility. There is moderate bilateral foraminal stenosis at the C3-4 level. At C4-5 there is moderate to severe bilateral foraminal stenosis with moderate central stenosis. At C5-6 there looks to be some spinal cord atrophy with myelomalacia. There is no significant foraminal stenosis at either C5-6 or C6-7. There is some metal artifact making examination of stenosis difficult. However there looks to be spinal fluid anterior and posterior to the spinal cord on the sagittal at both levels. No significant foraminal stenosis is seen at C7-T1.

Assessment
- Cervical disc degeneration C4 - C5
- Cervical spine stenosis
- Cervical radiculopathy at C5 nerve root

Plan
- **Cervicalgia**
  X-RAY Gillette/SPINE: Cervical / min 4

- Follow-up visit:

I explained to her that a pain pump could be a good option for her but that it is more of a salvage procedure when there is nothing else to offer. Right now, it seems like there 2/4

could possibly be some symptoms coming from the foraminal stenosis at the C4-5 level. My recommendation at this point is bilateral C4-5 transforaminal epidural steroid injections with Dr. Ptacek. She can call me 7 to 10 days later to let me know how it helps. If she gets significant relief with these injections but they are short-lived then we could possibly consider a fusion at the C4-5 level. If she gets no relief then I think we would have to assume that a lot of her symptoms are due to her spinal cord myelomalacia and problems related to her initial injury. At that point, I think that the pain pump could be an option. Right now it is not clear whether the intrathecal tubing will be threaded into the cervical spine or not. She does have moderate stenosis at the C4-5 level so I would have some concern about placing a space-occupying device in an area that is already stenotic. In addition, I need to review her CT scan to evaluate the OPLL. I explained to her that this is typically a progressive process but it is typically quite slow. If it is actually present at the this age, it is likely that it will become symptomatic at some point in her life but hopefully that is years to decades down the road. I can make further recommendations once I have the CT results and the results of the injection available. She can call me with any further questions or concerns.

Spinal injection procedure risks were discussed with the patient including transient increase in pain, transient neurologic deficit, infection, or blood loss. Typical side effects such as nausea, headache, dizziness, anxiety, or facial flushing were also discussed with the patient.

This appointment qualifies for moderate level of medical decision making as we are managing 1 more chronic illnesses with progression as well as recommending minor surgery with identified risk factors with her epidural injections.

Current Medication
- Baclofen 10 MG Oral Tablet 30 days, 0 refills
- HYDROcodone-Acetaminophen 7.5-325 MG Oral Tablet 4 days, 0 refills
- Pregabalin 50 MG Oral Capsule 30 days, 0 refills
- valACYclovir HCl 1 GM Oral Tablet 22 days, 0 refills

Social History
Tobacco use: Tobacco non-user.

Allergies
- Penicillinase Reaction: Hives / Urticaria

Practice Management
Use of tobacco assessment performed Review of medications documented.

Health Reminders
- Assess BMI satisfied 08/20/2025.
- Assess Tobacco Use satisfied 08/20/2025.

Robert J. Woodruff MD
Electronically signed by: Robert Woodruff M.D. Date: 08/20/2025 12:38

Electronically approved by: Robert Woodruff M.D. Date: 08/20/25 12:38

Campbell County Health

CCH RAD CT SCAN

Pulliam, Viktoriya A
MRN: 10132149, DOB: 12/6/1989, Legal Sex: F
Adm: 7/29/2025, D/C: 7/29/2025

07/29/2025 - CT Exam in Campbell County Health

Visit Information

Provider Information

Referring Provider

Thronburg, Sara Marie, FNP

Department

Name	Address	Phone	Fax
Campbell County Health	501 S Burma Ave Gillette WY 82716	307-688-1600	307-688-1640

Imaging Results

Imaging

CT CERVICAL SPINE POST MYELO (Final result)

CT CERVICAL SPINE POST MYELO Resulted 07/29/25 1025, Result status: Final result

Order status: Completed
Filed by: Edi, Rad Results n 07/29/25 1027
Accession number: 43454754
Resulted by: Baldwin, Tyler, MD
Performed: 07/29/25 0858 - 07/29/25 0906
Resulting lab: CCH PACS POWERSCRIBE

Narrative:
EXAM: CT cervical spine myelogram

REASON FOR EXAM: Cervicogenic headache

HISTORY: Cervicogenic headache; history of ACDF.

COMPARISON: MRI 6/17/2025

PROCEDURE: Multiple CT axial images of the cervical spine were obtained following fluoroscopically guided intrathecal injection of contrast. Reformatted coronal and sagittal images were performed. Please refer to fluoroscopic guided lumbar myelogram injection report for more details on the procedure.

Total accumulated CT dose for this study: 393 mGy.Cm
One or more of the following dose reduction techniques were used. Automated exposure control, adjustment of the mA and/or kV to patient size, use of iterative reconstruction techniques.

FINDINGS:

Craniocervical and atlantodental alignment are preserved. There is straightening of cervical lordosis. No spondylolisthesis. Vertebral body heights are maintained. No acute fractures are suspicious osseous lesions. Status post ACDF from C5-C7. Mature bony bridging of the vertebral bodies at these levels. No evidence of hardware loosening or complication.

There is intrathecal contrast within the spinal canal and extending into the basal cisterns. No intrathecal masses.

No potentially concerning extraspinal findings. The lung apices are clear.

Level by level findings are as follows:

C2-C3: No canal or foraminal stenosis. No degenerative changes.

C3-C4: No canal stenosis. Right greater than left uncovertebral arthropathy, causing mild-to-moderate right foraminal stenosis.

C4-C5: Disc-osteophyte complex formation, causing mild-to-moderate canal stenosis and mild flattening of the ventral cord. Mild bilateral uncovertebral arthropathy and left facet arthropathy, causing mild right foraminal narrowing.

C5-C6: OPLL, worse at the mid C6 level, which causes deformity of the ventral cord and moderate central canal stenosis. Mild bilateral facet arthropathy. No foraminal stenosis.

1/2

C6-C7: No canal or foraminal stenosis.

C7-T1: No canal or foraminal stenosis. Mild bilateral facet arthropathy.

Campbell County Health

CCH RAD CT SCAN

Pulliam, Viktoriya A
MRN: 10132149, DOB: 12/6/1989, Legal Sex: F
Adm: 7/29/2025, D/C: 7/29/2025

07/29/2025 - CT Exam in Campbell County Health

Visit Information

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Referring Provider
Thronburg, Sara Marie, FNP

Department

Name	Address	Phone	Fax
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Imaging Results

Imaging

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Order status: Completed
Filed by: Edi, Rad Results n 07/29/25 1027
Accession number: 43454754
Resulted by: Baldwin, Tyler, MD
Performed: 07/29/25 0858 - 07/29/25 0906
Resulting lab: CCH PACS POWERSCRIBE

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REASON FOR EXAM: Cervicogenic headache

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One or more of the following dose reduction techniques were used: Automated exposure control, adjustment of the mA and/or kV to patient size, use of iterative reconstruction techniques.

FINDINGS:

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C5-C6: OPLL, worse at the mid C6 level, which causes deformity of the ventral cord and moderate central canal stenosis. Mild bilateral facet arthropathy. No foraminal stenosis.

1/4

C6-C7: No canal or foraminal stenosis.

C7-T1: No canal or foraminal stenosis. Mild bilateral facet arthropathy.

07/29/2025 - CT Exam in Campbell County Health (continued)

Imaging Results (continued)

Impression
1. Status post ACDF from C5-C7 without evidence of hardware loosening or complication.
2. Disc-osteophyte complex at C4-C5 causing mild-to-moderate canal stenosis and mild flattening of the ventral cord.
3. OPLL at C6 causing deformity of the ventral cord and moderate central canal stenosis.
4. Mild-to-moderate right foraminal stenosis at C3-C4.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 7/29/2025 10:25 AM

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
747 - CCH_FUJI_RADPACS_PS	CCH PACS POWERSCRIBE	Unknown	Unknown	06/03/25 1510 - Present

Indications

Cervicogenic headache [G44.86 (ICD-10-CM)]

07/29/2025 - Diagnostic Radiology in Campbell County Health

Visit Information

Provider Information

Referring Provider

Thronburg, Sara Marie, FNP

Department

Name	Address	Phone	Fax
Campbell County Health	501 S Burma Ave Gillette WY 82716	307-688-1600	307-688-1640

Imaging Results

Imaging

XR MYELOGRAM CERVICAL (Final result)

XR MYELOGRAM CERVICAL Resulted: 07/29/25 0941, Result status: Final result

Order status: Completed
Filed by: Edi, Rad Results n 07/29/25 0943
Accession number: 43454850
Narrative:
Resulted by: Baldwin, Tyler, MD
Performed: 07/29/25 0811 - 07/29/25 0850
Resulting lab: CCH PACS POWERSCRIBE

EXAM: Myelogram

REASON FOR EXAM: Cervicogenic headache

HISTORY: Cervicogenic headache

COMPARISON: None

PROCEDURE: Procedure as well as its risks were discussed with the patient. Verbal and written consent was obtained. The lumbar region was prepped and draped in the standard sterile fashion. 1% lidocaine was used for local anesthesia. Under fluoroscopic guidance, a 22 - gauge needle was placed intrathecally using an interspinous approach at L3-L4. Subsequently, 12 mL of Isovue-M 200 was placed intrathecally. The needle was then removed and a dressing applied. The patient tolerated the procedure well. There were no immediate postprocedural complications. Post lumbar puncture instructions provided to the patient.

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Imaging Results (continued)

Total fluoroscopic time: 0.3 minutes

RAK: 2.9 mg

Impression:

Status post fluoroscopic myelogram prior to CT.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 7/29/2025 9:41 AM

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
747 - CCH_FUJI_RADPACS_PS	CCH PACS POWERSCRIBE	Unknown	Unknown	06/03/25 1510 - Present

Indications

Cervicogenic headache [G44.86 (ICD-10-CM)]

CAMPBELL COUNTY HEALTH

Outpatient Imaging
508 Stocktrail Ave.
Suite C, North Entrance
Gillette, WY 82716
Phone #: 307.688.1600
Fax #: 307.688.1640

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00110103603 Loc: WORIRAD
DOB: 12/06/1989 Age/Sex: 35 / F
Date of Service: 06/17/25
Attending Prov: Sara M Thronburg FNP
Ordering Prov: Sara M FNP Thronburg
Acc. #(s): A0000598307
CPT: 72141

MRI Report

cc: Doctor,No
Thronburg,Sara M FNP~

EXAM: Cervical spine MRI without contrast

REASON FOR EXAM: CERVICAL DISC DEGENERATION

HISTORY: Neck and right arm numbness. History of fusion in 2020.

COMPARISON: Brachial plexus MRI 6/2/2025

PROCEDURE: Multiplanar multisequence MRI images of the cervical spine were obtained including sagittal T1, T2 and STIR sequences, axial 3D TSE and oblique T2 TFE sequences from C3 through T1 at 3-4 mm intervals without contrast. Field strength 1.5 Tesla.

FINDINGS:

Craniocervical and atlantodental alignment are preserved. Straightening of cervical lordosis. No spondylolisthesis. Vertebral body heights are maintained. Status post ACDF from C5-C7. No evidence of hardware complication. No bone marrow edema or suspicious osseous lesions.

The imaged posterior fossa structures are unremarkable.

Small focus of high T2 signal within the left ventral cord at the inferior C5 level (series 9 image 29).. Just inferior to this, at the C5-C6 level, there are punctate foci of T2 hyperintense signal within the left and right central cord (series 9 image 32). No evidence of cord edema. No additional signal abnormalities within the cervical cord.

No potentially concerning extraspinal findings.

Level by level findings:

C2 - C3: No canal or foraminal stenosis.

C3 - C4: No canal stenosis. Bilateral uncovertebral and facet arthropathy, causing mild-moderate right and mild left foraminal narrowing.

C4 - C5: Posterior disc-osteophyte complex, causing mild canal stenosis. Bilateral uncovertebral and facet arthropathy, causing moderate right and mild left foraminal stenosis.

C5 - C6: At the C6 level, there is a central signal abnormality along the ventral thecal sac, causing moderate canal stenosis and deforming the ventral cord. No foraminal stenosis.

C6 - C7: No canal or foraminal stenosis.

C7 - T1: No canal or foraminal stenosis. Mild bilateral facet arthropathy and mild disc bulge.

ORDER #: 0617-0013 MR/MRI cervical spine wo con
IMPRESSION:
1. Small foci of high T2 signal within the left ventral cord at the C5 level and within the left and right central cord at the C5-C6 level, favored to reflect myelomalacia.
2. Moderate canal stenosis at the C6 level, possibly from OPLL.
3. Moderate right C5 foraminal stenosis.
4. Additional findings as described.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 6/17/2025 2:41 PM

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Dictated/Signed By: <Electronically signed by Tyler J Baldwin MD>06/17/25 1441

DD/DT: 06/17/25 1425
TD/TT: 06/17/25 1425 Transcriptionist: BTJ

CAMPBELL COUNTY HEALTH

Outpatient Imaging
508 Stocktrail Ave.
Suite C, North Entrance
Gillette, WY 82716
Phone #: 307.688.1600
Fax #: 307.688.1640

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct V00110103603 Loc: WORIRAD
DOB: 12/06/1989 Age/Sex: 35 / F
Date of Service: 06/17/25
Attending Prov: Sara M Thronburg FNP
Ordering Prov: Sara M FNP Thronburg
Acc. #(s): A0000598307
CPT 72141

MRI Report

cc: Doctor,No
Thronburg,Sara M FNP~

EXAM: Cervical spine MRI without contrast

REASON FOR EXAM: CERVICAL DISC DEGENERATION

HISTORY: Neck and right arm numbness. History of fusion in 2020.

COMPARISON: Brachial plexus MRI 6/2/2025

PROCEDURE: Multiplanar multisequence MRI images of the cervical spine were obtained including sagittal T1, T2 and STIR sequences, axial 3D TSE and oblique T2 TFE sequences from C3 through T1 at 3-4 mm intervals without contrast. Field strength 1.5 Tesla.

FINDINGS:

Craniocervical and atlantodental alignment are preserved. Straightening of cervical lordosis. No spondylolisthesis. Vertebral body heights are maintained. Status post ACDF from C5-C7. No evidence of hardware complication. No bone marrow edema or suspicious osseous lesions.

The imaged posterior fossa structures are unremarkable.

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C4 - C5: Posterior disc-osteophyte complex, causing mild canal stenosis. Bilateral uncovertebral and facet arthropathy, causing moderate right and mild left foraminal stenosis.

C5 - C6: At the C6 level, there is a central signal abnormality along the ventral thecal sac, causing moderate canal stenosis and deforming the ventral cord. No foraminal stenosis.

C6 - C7: No canal or foraminal stenosis.

Additional copy
Imaging 0617-0079

Campbell County Health Outpatient Imaging
Patient name: PULLIAM,VIKTORIYA A
DOB: 12/06/1989
Account #: V00110103603

2

C7 - T1: No canal or foraminal stenosis. Mild bilateral facet arthropathy and mild disc bulge.

ORDER #: 0617-0013 MR/MRI cervical spine wo con
IMPRESSION:
1. Small foci of high T2 signal within the left ventral cord at the C5 level and within the left and right central cord at the C5-C6 level, favored to reflect myelomalacia.
2. Moderate canal stenosis at the C6 level, possibly from OPLL.
3. Moderate right C5 foraminal stenosis.
4. Additional findings as described.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 6/17/2025 2:41 PM

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Dictated/Signed By: <Electronically signed by Tyler J Baldwin MD>06/17/25 1441

DD/DT: 06/17/25 1425
TD/TT: 06/17/25 1425 Transcriptionist: BTJ

Additional copy
Imaging 0617-0079

CAMPBELL COUNTY HEALTH

Outpatient Imaging
508 Stocktrail Ave.
Suite C, North Entrance
Gillette, WY 82716
Phone #: 307.688.1600
Fax #: 307.688.1640

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00110103603 Loc: WORIRAD
DOB: 12/06/1989 Age/Sex: 35 / F
Date of Service: 06/17/25
Attending Prov: Sara M Thronburg FNP
Ordering Prov: Sara M FNP Thronburg
Acc. #(s): A0000598307
CPT: 72141

MRI Report

cc: Doctor,No
Thronburg,Sara M FNP~

EXAM: Cervical spine MRI without contrast

REASON FOR EXAM: CERVICAL DISC DEGENERATION

HISTORY: Neck and right arm numbness. History of fusion in 2020.

COMPARISON: Brachial plexus MRI 6/2/2025

PROCEDURE: Multiplanar multisequence MRI images of the cervical spine were obtained including sagittal T1, T2 and STIR sequences, axial 3D TSE and oblique T2 TFE sequences from C3 through T1 at 3-4 mm intervals without contrast. Field strength 1.5 Tesla.

FINDINGS:

Craniocervical and atlantodental alignment are preserved. Straightening of cervical lordosis. No spondylolisthesis. Vertebral body heights are maintained. Status post ACDF from C5-C7. No evidence of hardware complication. No bone marrow edema or suspicious osseous lesions.

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No potentially concerning extraspinal findings.

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C4 - C5: Posterior disc-osteophyte complex, causing mild canal stenosis. Bilateral uncovertebral and facet arthropathy, causing moderate right and mild left foraminal stenosis.

C5 - C6: At the C6 level, there is a central signal abnormality along the ventral thecal sac, causing moderate canal stenosis and deforming the ventral cord. No foraminal stenosis.

C6 - C7: No canal or foraminal stenosis.

Department's copy
Imaging 0617-0079

Campbell County Health Outpatient Imaging 2
Patient name: PULLIAM,VIKTORIYA A
DOB: 12/06/1989
Account #: V00110103603

C7 - T1: No canal or foraminal stenosis. Mild bilateral facet arthropathy and mild disc bulge.

ORDER #: 0617-0013 MR/MRI cervical spine wo con
IMPRESSION:
1. Small foci of high T2 signal within the left ventral cord at the C5 level and within the left and right central cord at the C5-C6 level, favored to reflect myelomalacia.
2. Moderate canal stenosis at the C6 level, possibly from OPLL.
3. Moderate right C5 foraminal stenosis.
4. Additional findings as described.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 6/17/2025 2:41 PM

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Dictated/Signed By: <Electronically signed by Tyler J Baldwin MD>06/17/25 1441

DD/DT: 06/17/25 1425
TD/TT: 06/17/25 1425 Transcriptionist: BTJ

Department's copy
Imaging 0617-0079

CAMPBELL COUNTY HEALTH

Campbell Co. Memorial Hospital
501 S. Burma Ave.
P.O. Box 3011
Gillette, WY 82717
Phone #: 307.688.1600
Fax #: 307.688.1640

MRI Report

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00109957183 Loc: ORAD
DOB: 12/06/1989 Age/Sex: 35 / F
Date of Service: 06/02/25
Attending Prov: Hans L Carlson MD
Ordering Prov: Hans L Carlson, MD
Acc. #(s): A0000594623
CPT: 71552

cc: Carlson,Hans L MD
Doctor,No ~

EXAM: Brachial plexus MRI with and without contrast

REASON FOR EXAM: BRACHIAL PLEXOPATHY

HISTORY: Left-sided weakness. Underwent C5-C7 fusion in November of 2022.

COMPARISON: None available at the time of dictation

PROCEDURE: Multiplanar multisequence MRI images of the brachial plexus were obtained with and without contrast utilizing 7 mL of Vueway
Field strength 1.5 Tesla.

FINDINGS:

There are partially imaged post-surgical changes of surgical fusion from C5-C7. There appears to be canal stenosis at C5 and C6, with abnormal high T2 signal within the left ventral cord at the C5 level (series 8 image 12), and throughout the cord at the C6 level (series 8 image 17)

No signal abnormalities seen in the region of the brachial plexus nerve roots, trunks, divisions, cords, and branches. No areas of abnormal enhancement after contrast administration.

The lung apices are clear.

ORDER #: 0602-0010 MR/MRI brachial plexus wo/w con
IMPRESSION:
1. Normal MR appearance of the left brachial plexus.
2. Finding suggestive of canal stenosis in the lower cervical spine, with suspected areas of high T2 signal within the lower cervical cord, which could reflect myelomalacia or cord edema. Further evaluation with cervical spine MRI recommended.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 6/2/2025 2:54 PM

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Dictated/Signed By: <Electronically signed by Tyler J Baldwin MD>06/02/25 1454

DD/DT: 06/02/25 1442
Additional copy
Imaging 0602-0111

Campbell County Health Campbell Co. Memorial Hospital 2
Patient name: PULLIAM,VIKTORIYA A
DOB: 12/06/1989
Account #: V00109957183

TD/TT: 06/02/25 1442 Transcriptionist: BTJ

Additional copy
Imaging 0602-0111

2/3

BEST COPY

CAMPBELL COUNTY HEALTH

Campbell Co. Memorial Hospital
501 S. Burma Ave.
P.O. Box 3011
Gillette, WY 82717
Phone #: 307.688.1600
Fax #: 307.688.1640

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00109957183 Loc: ORAD
DOB: 12/06/1989 Age/Sex: 35 / F
Date of Service: 06/02/25
Attending Prov: Hans L Carlson MD
Ordering Prov: Hans L Carlson, MD
Acc. #(s): A0000594623
CPT: 71552

MRI Report

cc: Carlson,Hans L MD
Doctor,No ~

EXAM: Brachial plexus MRI with and without contrast

REASON FOR EXAM: BRACHIAL PLEXOPATHY

HISTORY: Left-sided weakness. Underwent C5-C7 fusion in November of 2022.

COMPARISON: None available at the time of dictation

PROCEDURE: Multiplanar multisequence MRI images of the brachial plexus were obtained with and without contrast utilizing 7 mL of Vueway
Field strength 1.5 Tesla.

FINDINGS:

There are partially imaged post-surgical changes of surgical fusion from C5-C7. There appears to be canal stenosis at C5 and C6, with abnormal high T2 signal within the left ventral cord at the C5 level (series 8 image 12), and throughout the cord at the C6 level (series 8 image 17)

No signal abnormalities seen in the region of the brachial plexus nerve roots, trunks, divisions, cords, and branches. No areas of abnormal enhancement after contrast administration.

The lung apices are clear.

ORDER #: 0602-0010 MR/MRI brachial plexus w/o/w con
IMPRESSION:
1. Normal MR appearance of the left brachial plexus.
2. Finding suggestive of canal stenosis in the lower cervical spine, with suspected areas of high T2 signal within the lower cervical cord, which could reflect myelomalacia or cord edema. Further evaluation with cervical spine MRI recommended.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 6/2/2025 2:54 PM

Some sections of this report may have been generated using a voice to text program. Every effort is made to correct errors, if mistakes are found they should be taken in context.

Dictated/Signed By: <Electronically signed by Tyler J Baldwin MD>06/02/25 1454

DD/DT: 06/02/25 1442
Additional copy
Imaging 0602-0111

1/3

2

Campbell County Health Campbell Co. Memorial Hospital
Patient name: PULLIAM,VIKTORIYA A
DOB: 12/06/1989
Account #: V00109957193

TD/TT: 05/02/25 1442 Transcriptionist: BTJ

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Imaging 0602-0111

2/3

CAMPBELL COUNTY HEALTH

Campbell Co. Memorial Hospital
501 S. Burma Ave.
P.O. Box 3011
Gillette, WY 82717
Phone #: 307.688.1600
Fax #: 307.688.1640

CAT Scan Report

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00109145839 Loc: ER10
DOB: 12/06/1989 Age/Sex: 34 / F
Date of Service: 08/14/24
Attending Prov:
Ordering Prov: Justino N Dalio, DO
Acc. #(s): A0000529860
CPT: 74176

cc: Dalio,Justino N DO
Doc,ED
Doctor,No ~

EXAM: CT abdomen and pelvis without contrast

REASON FOR EXAM: RLQ pain

HISTORY: Emergency department, initial

COMPARISON: None

PROCEDURE: Multiple CT axial images of the abdomen and pelvis were obtained without contrast. Reformatted coronal and sagittal images were performed.

Total accumulated CT dose for this study: 400.55 (mGy.cm)
One or more of the following dose reduction techniques were used: Automated exposure control, adjustment of the mA and/or kV to patient size, use of iterative reconstruction techniques.

FINDINGS:

Lung bases: Prominent bullous emphysematous changes are present at the right lung base with bibasilar atelectasis.

Liver: Norma morphology. No focal lesions.

Gallbladder/bile ducts: No stones or biliary ductal dilatation.

Pancreas: No ductal dilatation or peripancreatic stranding.

Spleen: No splenomegaly.

Adrenal glands: No nodules.

Kidneys: Bilateral nephrolithiasis measuring up to approximately 3 mm without evidence of obstructing stones or hydronephrosis. Probable 8 mm cyst at the anterior left renal cortex.

Bladder: No wall thickening.

Pelvic organs: Status post hysterectomy. No large adnexal cyst or mass is identified on this unenhanced examination.

Bowel/peritoneum: No bowel obstruction or abnormal bowel wall thickening. Normal appendix. No free air or fluid in the abdomen or pelvis.

Vessels: No abdominal aortic aneurysm. No calcified plaque.

Lymph nodes: No adenopathy.

Bones/soft tissues: L4-L5 degenerative disc disease.

ORDER #: 0814-0039 CT/CT abdomen pelvis wo IV con
IMPRESSION:

1.Unremarkable appendix.
2.No evidence of obstructing stones or hydronephrosis.
3.Bilateral nephrolithiasis.
4.Bullous emphysematous changes at the right lung base.
5.Additional chronic findings as above.

Some sections of this report may have been generated using a voice to text program. Every effort is made to correct errors, if mistakes are found they should be taken in context.

Dictated/Signed By: <Electronically signed by Paul W Rigsby DO>08/14/24 1633

DD/DT: 08/14/24 1629
TD/TT: 08/14/24 1629 Transcriptionist: RPW

CHEYENNE RADIOLOGY

BLACK ROCK ADVANCED MEDICAL IMAGING LLC

Viktoriya Pullia
DOB: 12-06-1989

Ref Physician: Angelo Santiago, Md
Exam Date: 05-08-2024

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MR - MRI BRAIN W WO

Reason for exam: Muscle weakness

Technique: Axial pre and post, sagittal and coronal post gadolinium T1, axial fast spin echo, FLAIR, diffusion gradient-echo sequences were performed through the brain on a 1.5 Tesla scanner.

Contrast: 7 cc Gadavist IV

Findings:

Cerebrum: No abnormal signal is identified. There is no evidence of any mass, edema, mass effect or midline shift. No signs of hemorrhage or extra-axial fluid collections. The arteries at the base of the brain are unremarkable.

Midline structures: Intact.

Posterior fossa: Tonsillar ectopia measures 4 mm below the foramen magnum. No evidence of tonsillar or medullary compression. No abnormal signal or mass effect identified.

Gradient-echo/diffusion sequences: Unremarkable. No abnormal signal is apparent. No evidence of ischemia or infarct.

Enhancement pattern: Normal.

Petrous bones and IACs: No masses or destruction. Normal mastoid sinuses.

Sinuses: Normal. No evidence of sinusitis.

Extracranial structures: Unremarkable.

Impression: Low-lying tonsils of the cerebellum, of questionable clinical significance. Otherwise normal study.

Dictated on: 5/8/2024 9:30 AM

This report has been electronically signed and approved by the interpreting Radiologist.

PATIENT NAME:POLLIA, VIKTORIYA
DOB:12/06/1989 MRNO:40023571

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Electronically Signed
John Wright, M.D.
2024-05-08 09:39:32

Cheyenne Radiology Group sincerely appreciates the opportunity to participate in this patients care. Please do not hesitate to contact myself, or one of my colleagues, at 307-634-7711 if we may be of further assistance.

2/3

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Summit Medical Center
- 6350 E 2nd St.
- Casper,WY 82609
- 3072326600
- Summitmedicalcasper.com
- RADIOLOGY REPORT

NAME: PULLIAM, VIKTORIYA
PATIENT ID: 16237
DOB: 12/6/1989
GENDER: F

EXAM: CT MYELOGRAM CERVICAL
ACCESSION: 08009510000100
DATE OF SERVICE: 3/28/2024
PROVIDER: THRONBURG, SARA

EXAMINATION:
Cervical spine CT myelogram

HISTORY: Neck pain.

COMPARISON: Cervical spine MRI dated 09/30/2021

TECHNIQUE:

Axial CT of the cervical spine after the administration of intrathecal contrast material. Sagittal and coronal reformats.

One of the following dose optimization techniques was utilized in the performance of this exam: Automated exposure control; adjustment of the mA and/or kV according to the patient's size; or use of an iterative reconstruction technique. Specific details can be referenced in the facility's radiology CT exam operational policy.

CONTRAST: See procedure note for contrast details.

FINDINGS:

Alignment: Normal.

Cranio-cervical junction: Negative.

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Vertebral bodies: Negative.

Posterior elements: Negative.

Hardware: Anterior discectomy and fusion hardware at C5-C6 and C6-C7. No perihardware lucency.

Soft tissues: Solid fusion from C5 to C7 with mild surrounding degenerative changes.

Visualized upper chest: Negative.

Disc Spaces:

C1-2: Negative.

C2-3: Negative.

C3-4: Mild disc bulge and uncovertebral hypertrophy. No significant spinal canal stenosis. Mild neural foraminal stenosis. No significant change.

C4-5: Mild disc space narrowing and disc bulge. Anterior disc osteophytes. Mild spinal canal and neural foraminal stenosis. No significant change.

C5-6: Solid interbody fusion. No significant stenosis. Postsurgical changes are new and stenosis is decreased compared with 09/30/2021.

C6-7: Solid interbody fusion. No significant stenosis. Postsurgical changes are new and stenosis is decreased compared with 09/30/2021.

C7-T1: Small central disc protrusion with no significant stenosis. No significant change.

IMPRESSION:

1. Multilevel degenerative disc disease and facet hypertrophy with anterior discectomy and fusion at C5-C6 and C6-C7.

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2. Solid fusion at C5-C6 and C6-C7. No perihardware lucency.

Report E-Signed By: Jason Paul Doye, MD, MD at 3/28/2024 2:57 PM
WSN:AMICRAD13

3/4

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Patient: **Pullman, Viktoriya** Sex: Female DOB: 12/06/1989
Exam: MRI C-Spine w/o (EMIG72141) Acc# 311451
DOS: 01/18/2024 04:46 PM

Ref. Phys: John Mansell, MD
3100 W LAKEWAY RD SUITE 3, GILLETTE,
WY 82718-6372

* * *Final Report* * *

DATE OF EXAM: Jan 18 2024 4:46PM MST

E2M 0482 - MRI CERVICAL SPINE WO CONTRAST / ACCESSION # 150307963

PROCEDURE REASON: M961

* * * * Physician Interpretation * * * *

RESULT: HISTORY:

34-year-old with new onset of tingling.

TECHNIQUE:

Routine MR imaging through the cervical spine is performed without IV contrast administration. There are no prior MRIs for comparison. T2-weighted left and right sagittal oblique images are also obtained. This study is correlated with plain radiographs 01/18/2024.

RESULT:

Counting reference: Craniocervical junction. Anatomic Variants: None.

Coronal localizer images reveal a very mild levoscoliosis centered at the cervicothoracic junction.

Sagittal images reveal a patient to be status post ACDF at C5/C6 and C6/C7. There is some magnetic susceptibility artifact at these levels. The disc space at C4/C5 is mildly narrowed. The cervical vertebral bodies are normal in height and demonstrate normal bone marrow signal

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intensity. There is increased T2-weighted signal intensity bilaterally within the substance of the spinal cord at the level of C5/C6.

There are no prevertebral masses.

Axial images at C2/C3 reveal no compression of the thecal sac nor the spinal cord. The neural foramina are patent.

At C3/C4, there is mild central stenosis due to a bulging disc. There is no cord compression. The left foramen is patent. There is mild right foraminal narrowing due to uncovertebral joint hypertrophy.

At C4/C5, there is mild to moderate central stenosis and mild cord flattening due to a diffusely bulging disc. There is mild left and moderate right foraminal narrowing due to facet and uncovertebral joint hypertrophy and extension of disc osteophyte complexes into the neural foramina, particularly on the right.

At C5/C6, there are postoperative changes. There is mild central stenosis but no cord compression. The left foramen is patent. There is minimal right foraminal narrowing due to uncovertebral joint hypertrophy.

At C6/C7, there are postoperative changes. There is mild central stenosis but no cord compression. The neural foramina are patent.

At C7/T1, there is mild to moderate central stenosis and mild cord flattening due to a bulging disc, eccentric towards the right. The neural foramina are patent.

The cerebellar tonsils project 6.5 mm inferior to the foramen magnum.

IMPRESSION:

1. Postoperative changes at C5/C6 and C6/C7.

2. Multilevel central stenosis involving the C3/C4 through C7/T1, inclusive, levels due to bulging disks, osteophytic spurring, and disc osteophyte complexes.

3. Mild cord flattening at several levels as noted above.

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4. Cord myelomalacia at C5/C6, likely related to chronic cord compression.

5. Neural foraminal narrowing due to facet and uncovertebral joint hypertrophy.

6. Low position of the cerebellar tonsils relative to the foramen magnum, likely reflecting a Chiari I malformation. A brain MRI can be considered to completely exclude intracranial hypotension as a cause for the low position of the cerebellar tonsils.

7. No cervical cord syrinx.

Transcribe Date/Time: Jan 19 2024, 8:37A

Dictated by: JEFFREY SPREITZER, MD

This examination was interpreted and the report reviewed and electronically signed by:
JEFFREY SPREITZER, MD on Jan 19 2024 8:41AM EST

Thank you for allowing us to participate in the care of your patient.
Should there be any questions regarding this interpretation, please call 307-682-1779.
If you are unable to reach us at the number above,
please feel free to contact Cleveland Clinic eRadiology at 866-328-6380.

Electronically Signed By:
Jeffrey J. Spreitzer at 01/19/2024 06:44

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Elevation Medical
IMAGING

Patient: **Pullman, Viktoriya** Sex: Female DOB: 12/06/1989
Exam: Xray C-Spine 4-5 views (EMIG72050) Acc# 311452
DOS: 01/18/2024 04:35 PM

Ref. Phys: John Mansell, MD
3100 W LAKEWAY RD SUITE 3, GILLETTE,
WY 82718-6372

Final Report

DATE OF EXAM: Jan 18 2024 4:35PM MST

E2X 5311 - XR CERVICAL 4V AP/LAT/OBL / ACCESSION # 150307964

PROCEDURE REASON: M961

**** Physician Interpretation ****

RESULT: HISTORY: M961. Left arm pain prior surgery.

TECHNIQUE: XR CERVICAL 4V AP/LAT/OBL
Laterality: Not applicable
Number of different views (projections): 4

COMPARISON: None

RESULT:

Counting reference: Craniocervical junction.

Postoperative changes of anterior plate and screw fusion hardware involving C5-C7 with interbody cage devices. Straightening of the cervical spine. Small endplate osteophytes greatest involving C4-C5. Mild disc space narrowing at C4-C5. No abnormal motion is detected on flexion or extension views. Prevertebral soft tissues are within normal limits.

IMPRESSION:

Postoperative and degenerative changes of the cervical spine.

Elevation Medical Imaging Gillette • 913 Boxelder RD., Gillette, WY 82718 • Phone: (307) 682-1779

CAMPBELL COUNTY HEALTH

Campbell Co. Memorial Hospital
501 S. Burma Ave.
P.O. Box 3011
Gillette, WY 82717
Phone #: 307.688.1600
Fax #: 307.688.1640

X-Ray Report

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00106504624 Loc: ORAD
DOB: 12/06/1989 Age/Sex: 32 / F
Date of Service: 04/05/22
Attending Prov: Eric B Schubert MD
Ordering Prov: Eric B Schubert, MD
Acc. #(s): A0000362604
CPT: 72050

cc: Hawk,Amy E APRN
Schubert,Eric B MD~

EXAM: Cervical spine x-ray

REASON FOR EXAM: ARTHRODESIS STATUS

HISTORY: Outpatient. Status post surgery in November 2021.

PROCEDURE: AP, lateral, flexion and extension

FINDINGS:

Comparison made with the cervical spine radiograph from January 31, 2022.

Anterior fusion redemonstrated at the C5-C7 levels. Hardware appears intact without evidence of loosening. No significant interval bony bridging is identified. Straightening of the cervical spine otherwise alignment is maintained. No evidence of instability during flexion and extension. Remaining disc spaces and vertebral body heights appear preserved. Prevertebral soft tissues are within normal limits.

ORDER #: 0405-0012 XR/XR cervical spine bend 4/5V
IMPRESSION:

Status post ACDF at C5-C7 similar to the prior.

Some sections of this report may have been generated using a voice to text program. Every effort is made to correct errors, if mistakes are found they should be taken in context.

Dictated/Signed By: <Electronically signed by Paul W Rigsby DO>04/05/22 0923

DD/DT: 04/05/22 0920
TD/TT: 04/05/22 0920 Transcriptionist: RPW

Additional copy
Imaging 0405-0028

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

09/10/2025 Tyler Ptacek MD

Chief Complaint

1. Here for Cervical Transforaminal epidural injection
2. had prior infusion in neck on going pain Galloway, Linda, M. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
3. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
4. Left arm is spasming with placing arm behind back
5. Feels "twitching" throughout entire body
6. Seen numerous neurologists
7. Taking numerous OTCs

History of Present Illness

Disability indexing:

Disability scale PDI =26 Date of administration:9/10/25.

Injection:

The patient is here for a procedure for severe pain control Epidural for radiating pain. Feels well except for the pain. Denies F/C/N/V/D. Patient continues with severe neck pain radiating to the arm. Pain is severe >7/10 and debilitating - affecting ADL's and sleep. The patient continued conservative cares including self-directed HEP, Stretching, mindfulness, and physician directed HEP for the last 6+ weeks without meaningful relief of the severe pain.

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index=28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Vital Signs

Examination

Pain Exam:

General Mld to moderate distress due to pain, Respiratory drive normal, Alert and oriented x3, No gross motor deficits, Slow ambulation due to pain.
Trigger points taut bands of muscles with clear trigger points that replicate pain with pressure and restrict range of motion at the bilateral cervical paraspinals and trapezius

head and neck region shows limited movement and resistance to stretching muscles that is producing an abnormal posture. The affected muscles are: splenius capitis, sternocleidomastoid, scalene complex, levator scapulae, trapezius

SHe has rigidity left upper extremity.
Cervical radiculitis bilateral Severe guarding of the neck
TTP at the cervical paraspinal and trapezius musculature
Decreased ROM due to severe pain
Spurling's positive bilaterally
Reflexes 2/4 and symmetrical UE's
Normal strength testing of the UE's exception at the bilateral traps.

1/4

Assessments

1. Cervical spondylosis with radiculopathy - M47.22 (Primary)
2. Cervical spinal stenosis - M48.02
3. Cervicalgia - M54.2
4. Chronic pain syndrome - G89.4

Procedures

Cervical transforaminal epidural steroid injection (TESI):

Pre/Post operative diagnosis Cervical radiculitis, neck pain.
Surgeon Tyler A Ptacek, MD.
Indications The patient has radicular pain that has resulted in a functional impairment for at least 6 weeks. Pain is at least 3/10 and interferes with at least 2 ADL's. There is advanced imaging with nerve root compression/stenosis that correlates with clinical findings.
Procedure After informed consent of the potential risks: bleeding/infection/injury of neurovascular structures/reaction to medication, the benefits, and the alternatives, the patient was placed in a lateral position. Sterile prep and drape were completed. Under fluoroscopic imaging, the LEFT C4/5 level was identified from above and below. A 3.5 inch x 25g spinal needle was then advanced using fluoroscopic guidance as well as tactile sensation until the needle was ascertained to be in the lateral foramen. After negative aspiration, iodinated contrast, omnipaque 240 was then injected and needle placement confirmed with fluoroscopic imaging. A total of 0.5cc of contrast was injected. Subsequently, after negative aspiration, 5mg Decadron PF mixed with 0.5cc 1% Lidocaine was injected. The needle was withdrawn and sterile dressing applied.

Next under fluoroscopic imaging, the RIGHT C4/5 level was identified from above and below. A 3.5 inch x 25g spinal needle was then advanced using fluoroscopic guidance as well as tactile sensation until the needle was ascertained to be in the lateral foramen. After negative aspiration, iodinated contrast, omnipaque 240 was then injected and needle placement confirmed with fluoroscopic imaging. A total of 0.5cc of contrast was injected. Subsequently, after negative aspiration, 5mg Decadron PF mixed with 0.5cc 1% Lidocaine was injected. The needle was withdrawn and sterile dressing applied.

The patient was taken to the recovery room for the usual recovery time prior to discharge with written and verbal instructions per routine.
Fluoro images were permanently saved for future reference.
Pre-procedure pain score 3.
Post-procedure pain score 2
Percentage relief: 10.

Procedure Codes

- 64479 INJ FORAMEN EPIDURAL C/T, Modifiers: 50
- Q9966 LOCM 200-299MG/ML IODINE,1ML
- J1100 DEXAMETHASONE PHOSPHATE 1MG, Units: 10.00

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 09/13/2025 at 12:53 PM MDT

Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Account Financial History By Service Date

Black Hills Orthopedic & Spine Center, PC

Selections:

Service Dates: 11/21/2019 - 11/06/2025
Accounts: 329043
Activity Types: Charges, Payments, Adjustments, Transfers, Refunds

Type Date Legend:
Charges - Service Date, Credits - Post Date

Account Date	Type	Name / Description	Provider	Proc Code		Ref Date	Diagnosis	Units	Amount
329043		**Pulliam, Viktoriya**							
08/20/2025	PMT	1158958504			Charge from Pulliam, Viktoriya	08/20/2025			[illegible]
08/20/2025	CHG	Pulliam, Viktoriya	RJW	99204	Offic/outpt Visit E&m New Mod-Hi Se	08/21/2025	M48.02	1.00	[illegible]
08/20/2025	CHG	Pulliam, Viktoriya	RJW	72050	Rad Exam Spine Cerv; Mini 4 Views	08/21/2025	M48.02	1.00	[illegible]
09/04/2025	ADJ	[illegible]			[illegible]				[illegible]
09/04/2025	[illegible]	[illegible]			[illegible]				[illegible]
09/04/2025	PMT	825240000249369			Elec. Transfer from FIRST	09/03/2025			[illegible]
	Account Totals:	**PMT: 386.00**	**RFD: 0.00**	**XFR: 0.00**	**ADJ:**	**CHG:**	**531.00**	**2.00**	**0.00**
	Report Totals:	**PMT: 386.00**	**RFD: 0.00**	**XFR: 0.00**	**ADJ:**	**CHG:**	**531.00**	**2.00**	**0.00**